

Axcel® CEFACLOR-125 SUSPENSION

COMPOSITION:

Each 5ml contains
Cefaclor Monohydrate
equivalent to Cefaclor

125 mg

PRESENTATION:

Off white coloured granules which give a yellow to orange yellow coloured suspension with fruity flavour when reconstituted.

INDICATIONS:

Treatment of the following infections caused by susceptible organisms: otitis media, lower respiratory tract infections, pharyngitis and tonsillitis, urinary tract infection, skin and soft tissue infections.

Cefaclor is generally effective in the eradication of streptococci from the nasopharynx.

PHARMACOLOGY:

The presence of food may delay the absorption of cefaclor, but the total amount absorbed is unchanged. It is widely distributed in the body. It crosses the placenta and is excreted in low concentration in breast milk. It is rapidly excreted by the kidney. It may be slightly prolonged in renal failure. Cefaclor is well absorbed after oral administration to fasting subjects. Total absorption is the same whether the drug is given with or without food; however, when it is taken with food, the peak concentration achieved is 50 - 75% of that observed when the drug is administered to fasting subjects and generally appears from 3/4 - 1 hour later. Following administration of 250mg, 500mg and 1g doses to fasting subjects, average peak serum levels of approximately 7, 13 and 23mg/l, respectively were obtained within 30 - 60 minutes. Approximately 60 - 85% of the drug is excreted unchanged in the urine within eight hours greater portion being excreted within the first 2 hours. During this 8 hours period, peak urine concentrations following the 250mg, 500mg and 1g doses were approximately 600, 900 and 1900mg/ml, respectively. The serum half-life in normal subjects is 0.6 - 0.9 hour. In patients with reduced renal function, the serum half-life of cefaclor is slightly prolonged. In those with complete absence of renal function, the plasma half-life of the intact molecule is 2.3 - 2.8 hours. Excretion pathways in patients with

markedly impaired renal function have not been determined. Hemodialysis shortens the half-life by 25 - 30%.

In vitro tests demonstrated that the bactericidal action of cephalosporins results from inhibition of cell wall synthesis. While in vitro studies have demonstrated the susceptibility of most strains of the following organisms to cefaclor, clinical efficacy for infections other than those included in the indications and dosage & administration is unknown.

Aerobes, Gram Positive: Staphylococci, including coagulase-positive, coagulase-negative and penicillinase-producing strains, exhibit cross-resistance between cefaclor and methicillin; *Streptococcus pneumoniae*; *Streptococcus pyogenes*.

Aerobes, Gram-Negative: *Citrobacter diversus*; *Escherichia coli*; *Haemophilus influenza*, including β -lactamase-producing, ampicillin-resistant strains; *Klebsiella* sp; *Moraxella* (*Branhamella*) *catarrhalis*; *Neisseria gonorrhoeae*; *Proteus mirabilis*.

Anaerobes: *Bacteroides* sp (excluding *Bacteroides fragilis*); *Peptococcus niger*; *Peptostreptococcus* sp; *Propionibacterium* sp.

Note: Methicillin-resistant staphylococci and most strains of enterococci (*Enterococcus faecalis* and *Enterococcus faecium*) are resistant to cefaclor and other cephalosporins. Cefaclor is not active against most strains of *Enterobacter* and *Serratia* spp, *Morganella morganii*, *Proteus vulgaris* and *Providencia rettgeri*. It has no activity against *Pseudomonas* sp or *Acinetobacter* sp.

DOSAGE AND ADMINISTRATION:

To be administered orally.

Children: The usual recommended daily dose is 20mg/kg/day in divided doses administered 3 times daily. In more serious infections, otitis media and infections caused by less susceptible organisms 40mg/kg/day in divided doses are recommended, with a maximum dosage in 1g/day.

Child's Age & Weight	125mg / 5ml
	Cefaclor Suspension
<1 year (9kg)	2.5ml 3 times a day
1-5 year (9-18kg)	5ml 3 times a day
>5 years (18kg)	10 ml 3 times a day

Twice a day treatment option: for the treatment of otitis media and pharyngitis, the total daily dosage may be divided and administered every 12 hours.

Cefaclor may be administered in the presence of impaired renal function. Under such a condition, the dosage usually is unchanged.

Direction for mixing: Shake bottle well to loosen granules. Remove cap and Dose-Master™ adapter. Then, add some water to mix with the granules. After that, shake and top up to 60ml / 120ml mark. Place the Dose-Master™ adapter and cap. Shake the bottle well to mix the medicine properly.

CONTRAINDICATION:

Contraindicated in patient hypersensitive to Cefaclor or other cephalosporins.

PRECAUTION:

Cefaclor should be used with caution for patient with penicillin sensitivity. False positive urinary glucose and false positive Combs' test have been found during the treatment of Cefaclor.

Patients with renal impairment should be given with caution. Prolong use of cefaclor may result in the overgrowth of nonsusceptible organisms. Careful observation of patients is essential. Special precaution should be given to patients with history of gastrointestinal diseases, particularly colitis.

Serious and occasionally fatal hypersensitivity reactions (including anaphylactoid and severe cutaneous adverse reactions) have been reported in patients receiving therapy with beta-lactams. Before initiating therapy with Axcel Cefaclor-125 Suspension, careful inquiry should be made concerning previous hypersensitivity reactions to penicillins, cephalosporins, carbapenems or other beta-lactam agents. If an allergic reaction occurs, Axcel Cefaclor-125 Suspension must be discontinued immediately and appropriate alternative therapy instituted.

Use in Pregnancy and Lactation:

Cefaclor is widely distributed in the body. It crosses the placenta and small quantities are found in the milk of nursing mother. This drug should be used during pregnancy only if clearly needed. Caution should be exercised when it is administered to a nursing mother.

SIDE EFFECTS :

Gastro-intestinal: The most frequent side - effect has been diarrhoea. It is rarely severe enough to warrant cessation of therapy. Colitis, including rare instances of pseudomembranous colitis, has been reported. Nausea and vomiting have also occurred.

Hypersensitivity: Allergic reactions such as morbiliform eruptions, pruritus and urticaria have been observed. These reactions usually subside upon discontinuation of therapy. Serum sickness - like reactions (erythema multiforme minor, rashes or other skin manifestations accompanied by arthritis/arthralgia, with or without fever) have been reported. Lymphadenopathy and proteinuria are infrequent; there are no circulating immune complexes and no evidence of sequelae. Occasionally, solitary symptoms may occur, but do not represent a serum sickness - like reaction. Serum sickness - like reactions are apparently due to hypersensitivity and have usually occurred during or following a second (or subsequent) course of therapy with cefaclor. Such reactions have been reported more frequently in children than in adults. Signs and symptoms usually occur a few days after initiation of therapy and usually subside within a few days of cessation of therapy. Antihistamines and corticosteroids appear to enhance resolution of the syndrome. No serious sequelae have been reported.

There are rare reports of erythema multiforme major (Stevens - Johnson syndrome), toxic epidermal necrolysis, and anaphylaxis. Anaphylaxis may be more common in patients with a history of penicillin allergy. Anaphylactoid events may present as solitary symptoms, including angioedema, asthenia, oedema (including face and limbs), dyspnoea, paraesthesias, syncope, or vasodilatation.

Rarely, hypersensitivity symptoms may persist for several months.

Haematological: Eosinophilia, positive Coombs' tests and, rarely, thrombocytopenia. Transient lymphocytosis, leucopenia and rarely, haemolytic anaemia, aplastics anaemia, agranulocytosis and reversible neutropenia of possible clinical significance. See 'Drug Interactions'.

Hepatic: Transient hepatitis and cholestatic jaundice have been reported rarely, slight elevations in AST, ALT or alkaline phosphatase values.

Renal: Reversible interstitial nephritis has occurred rarely, also slight elevations in blood urea or serum creatinine or abnormal urinalysis.

Central nervous system: Reversible hyperactivity, agitation, nervousness, insomnia, confusion, hypertonia, dizziness, hallucinations and somnolence have been reported rarely.

Miscellaneous: Genital pruritus, vaginitis and vaginal moniliasis.

DRUG INTERACTIONS:

Rare cases of increase anticoagulant effect when cefaclor and oral anticoagulants were administered concomitantly. With other beta-lactam antibiotics, the renal excretion of cefaclor is inhibited by probenecid.

OVERDOSAGE AND TREATMENT:

The toxic symptoms of cefaclor may include: nausea, vomiting, epigastric distress and diarrhoea where the severity for the latter is doses related.

Activated charcoal is normally used for the treatment for overdose. Repeated doses of charcoal over time may hasten elimination of the drug that has been absorbed. Protect the patient's airway and support ventilation is important when employing charcoal or gastric emptying.

STORAGE:

Keep container well closed. Store below 30°C until reconstituted. After reconstitution, refrigerate between 2°C to 8°C and use within 7 days.

KEEP OUT OF REACH OF CHILDREN JAUHI DARI KANAK-KANAK

PACK QUANTITIES:

Available in 60ml and 120ml amber glass bottle.

Further information can be obtained from pharmacists, doctor and the manufacturer.

Product Registration Holder & Manufacturer



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